

1. Administrative & Study Identification

Full Study Title: A Randomized Open Label Phase II Study of Immune Checkpoint Inhibitor Combinations With Axitinib in Patients With Previously Untreated Locally Advanced Unresectable or Metastatic Renal Cell Carcinoma **Short Title/Acronym:** B043936 / Immune Checkpoint Inhibitor Combinations With Axitinib in RCC **Protocol Number:** B043936 **NCT Number:** NCT05805501 **Posting ID:** 661000798459730620 **Trial Status:** Active, Not Recruiting **Sponsor Name:** Hoffmann-La Roche (F. Hoffmann-La Roche Ltd) **Investigational Product:** Tobemstomig (R07247669, PD1xLAG3 bispecific antibody), Tiragolumab (anti-TIGIT), Axitinib (Inlyta), Pembrolizumab **Phase of Development:** Phase II **Medical Monitor:** F. Hoffmann-La Roche Ltd Clinical Operations

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy (Progression-Free Survival - PFS), safety, and pharmacokinetics of tobemstomig and axitinib, with or without tiragolumab, compared to standard-of-care pembrolizumab plus axitinib in patients with previously untreated locally advanced unresectable or metastatic clear-cell renal cell carcinoma (ccRCC). **Secondary Objectives:** To evaluate the Objective Response Rate (ORR), Overall Survival (OS), Duration of Response (DOR), and characterize the incidence and severity of Adverse Events (AEs).

2.2 Background & Rationale While immune checkpoint inhibitors and tyrosine kinase inhibitors (TKIs) like axitinib have improved outcomes in advanced RCC, cures remain rare and disease progression typically occurs within two years. To address this gap and combat T-cell exhaustion mediated by multiple checkpoint pathways, this study investigates combinations targeting novel immune checkpoints (LAG-3 and TIGIT) alongside PD-1 blockade and VEGFR inhibition.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Pembrolizumab + Axitinib) **Blinding:** Open-label **Randomization:** Randomized (3 Arms, Parallel Group)

3.2 Planned Enrollment & Duration Target **NS:** 199 participants **Number of Sites:** Multi-center (Global) **Estimated Study Start:** March 2023 **Estimated Primary Completion:** September 2024

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age $\geq$ 18 years. 2. Histologically confirmed clear-cell renal cell carcinoma (ccRCC) with or without sarcomatoid features. 3. Previously untreated, locally advanced unresectable or metastatic disease. 4. Measurable disease with at least one measurable lesion per RECIST v1.1. 5. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 or 1. 6. International Metastatic RCC Database Consortium (IMDC) risk intermediate (score of 1 or 2) or poor (score of 3-6).

4.2 Exclusion Criteria 1. Prior treatment for localized and/or metastatic RCC with systemic RCC-directed therapy, including T-cell costimulating or immune checkpoint blockade therapies. 2. Pregnant or breastfeeding women. 3. Inability to swallow a tablet or malabsorption syndrome. 4. Positive for HIV, hepatitis B, or hepatitis C virus (HCV). 5. Ongoing use or anticipated need for treatment with a strong CYP3A4/5 inhibitor or inducer. 6. Uncontrolled or symptomatic hypercalcemia requiring active intervention.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Arm A (Experimental): Tobemstomig (R07247669) + Axitinib Arm B (Experimental): Tobemstomig (R07247669) + Tiragolumab + Axitinib Arm C (Active Comparator): Pembrolizumab + Axitinib

Dosage/Frequency: Tobemstomig, Tiragolumab, and Pembrolizumab are administered every 3 weeks (Q3W) on Day 1 of each 21-day cycle. Axitinib is administered twice daily (BID). **Route of Administration:** Intravenous (IV) for Tobemstomig, Tiragolumab, and Pembrolizumab; Oral (PO) for Axitinib. **Duration of Treatment:** Until disease progression, unacceptable toxicity, or patient withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive oncology care. **Prohibited:** Strong CYP3A4/5 inhibitors or inducers; live attenuated vaccines; concurrent systemic anti-cancer therapies.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	-28 to 0	Screening: Informed Consent, Medical History, Tumor Imaging (CT/MRI), Baseline Labs, Eligibility Assessment
2	Day 1 of Cycle 1	Baseline: Randomization, First Dose Administration, PK/PD Sampling
3-10	Q3W (Day 1 of 21-day cycle)	Treatment Cycles: Drug Infusion (IV), Axitinib Dispensation (PO), Safety Labs, AE Assessment, Tumor Response Tracking
11	30 days post-last dose	End of Study

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-Free Survival (PFS) - defined as the time from randomization to the first documented occurrence of disease progression or death from any cause.

Measurement Method: Evaluated using Response Evaluation Criteria in Solid Tumors (RECIST) v1.1 via Investigator and/or Independent Central Review.

7.2 Secondary & Exploratory Endpoints 1. Objective Response Rate (ORR) 2. Overall Survival (OS) 3. Duration of Response (DOR) 4. Pharmacokinetics (PK) of Tobemstomig and Tiragolumab

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via HCP interview and laboratory assessments, specifically monitoring for overlapping immune-related adverse events (irAEs) and VEGFR-TKI toxicities (e.g., hypertension, hepatotoxicity). **Serious Adverse Events (SAEs):** Required reporting typically within 24 hours of site awareness. **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee to evaluate safety signals across the three arms periodically.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy outcome (PFS). Safety Analysis Set for all participants who receive at least one dose of study medication. **Sample Size**

Justification: 199 patients randomized to achieve sufficient statistical power to detect meaningful differences in PFS between the experimental arms and the control arm. **Handling of Missing**

Data: Standard survival analysis censoring logic for PFS and OS; Last Observation Carried Forward (LOCF) for relevant continuous variables if required.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Standard risk-based onsite and remote monitoring visits. **Audit Trail:** Electronic Data Capture (EDC) system with full audit trails, eCRF locking mechanisms, and query generation protocols.

11. Study Identifiers & Governance

Primary ID: B043936 **Registry Number:** NCT05805501 **Sponsor/Lead Organization:** Hoffmann-La Roche **Study Title:** Immune Checkpoint Inhibitor Combinations With Axitinib in RCC **Official Regulatory Title:** A Randomized Open Label Phase II Study of Immune Checkpoint Inhibitor Combinations With Axitinib in Patients With Previously Untreated Locally Advanced Unresectable or Metastatic Renal Cell Carcinoma

12. Clinical Framework (Renal Cell Carcinoma Specific)

Primary Condition: Clear-Cell Renal Cell Carcinoma (ccRCC)
Diagnosis Threshold: Histologically confirmed ccRCC, previously untreated, locally advanced unresectable or metastatic
Medical Methodology: Targeted Immunotherapy (PD-1/LAG-3/TIGIT blockade) combined with Tyrosine Kinase Inhibitor (VEGFR blockade)
Optimization Target: Maximize progression-free survival and T-cell reinvigoration in the tumor microenvironment.

13. Study Procedures & Methodology

Management Pathway: Standard oncology treatment pathway for advanced RCC
Education Protocol (HCP): Site initiation visits and continuous monitoring regarding immune-related AEs and multi-drug toxicity management.
Education Protocol (Patient): Informed consent process detailing the experimental nature of bispecific antibody therapies, dosing diaries for oral axitinib.
Quality Audit Mechanism: Clinical monitoring and source data verification processes.

14. Observation & Follow-Up Schedule

Total Duration: Follow-up until disease progression, death, or study termination.
Onsite Visit Cadence: Every 3 weeks (Day 1 of each 21-day cycle).
Remote Monitoring Frequency: Routine data monitoring per Institutional protocols.
Checklist Review Interval: Safety tracking at every cycle interval (Q3W).

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				